Which treatment this guide covers NCCP 00220a covers adults with locally advanced or metastatic NSCLC with an activating EGFR mutation. This is continuous treatment for advanced disease, not a fixed adjuvant course.

NCCP 00220 · VERSION 5

Gefitinib monotherapy

The usual dose is 250 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Make treatment fit into daily life

Take once daily with or without food, at about the same time. If missed, take when remembered unless less than 12 hours remain before the next dose; then skip it. Never double a dose. If swallowing is difficult, pharmacy can teach dispersion in non-carbonated water without crushing; drink within 60 minutes and rinse the glass. Ask your team what to do after vomiting; do not take an extra tablet without advice.

Before starting

Confirm the mutation, treatment setting, prescription, baseline tests, first review and emergency contact.

Taking the tablets

Take once daily with or without food, at about the same time. If missed, take when remembered unless less than 12 hours remain before the next dose; then skip it. Never double a dose. If swallowing is difficult, pharmacy can teach dispersion in non-carbonated water without crushing; drink within 60 minutes and rinse the glass. Ask your team what to do after vomiting; do not take an extra tablet without advice.

Supplies and visits

Arrange the next prescription before tablets run out. Keep your blood-test and clinic appointments. The oral regimen itself does not require an infusion line or pump.

If treatment is held

Ask why, what must recover and when review is due. Only the team authorises restarting or changing dose.

At treatment change

Agree ongoing symptom care, scans, contraception and follow-up.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00220 v5 · official regimen
- EMA Iressa product information
- NCCP SACT antiemetic guidance V6 (2025), oral tables
- NCCP NCIS antiemetic medicines V8 (2025); parenteral scope
- IPASS primary trial, Mok et al. 2009
- IPASS final OS and biomarker analysis, Fukuoka et al. 2011

Source reconciled; independent clinical, pharmacy and patient/PPI review pending